

model, and embracing all the chief particulars essential for the purposes of vital statistics, has been in operation.

Of the West Indian colonies, Jamaica, Barbadoes, Tobago, and Antigua, have some sort of imperfect registration, but it is chiefly conducted by the clergy, and deals almost exclusively with the Episcopalian part of the community. In British Honduras there is no registration at all. In Turk's Islands, the Bahamas, St. Vincent, Grenada, Montserrat, St. Kitt's, Nevis, Virgin Islands, and Dominica, a Registrar-General (in some cases the Colonial Secretary holds the office), with subordinate officers, have been appointed, and returns of births and deaths are annually made. The Registrar-General of Grenada has published a report for his first year of office (1866), giving tolerably complete returns, with a classification of diseases on Dr. Farr's system. In Trinidad there is apparently an imperfect registration.

At Gibraltar only births and marriages were registered at the date of our information, but it was in contemplation to include the deaths also. At Malta registration is conducted under the very precise and elaborate regulations of an Ordinance of 1862. Under clause 65 of that Ordinance, the physician or surgeon in attendance during the last illness is required to transmit to the registration officer a notice specifying the time, place, and cause of death.

At Sierra Leone, Gambia, and the Gold Coast, registration of some kind appears to be in operation. At the Cape of Good Hope a death register only is kept, but the "cause of death" is not recorded. St. Helena has a registration system framed after the English model; and in Ceylon, by an Ordinance of 1867, the system previously in force was greatly improved. In the Mauritius, registration is conducted on a similar plan to that adopted in France.

From this brief and hasty analysis it may be gathered that in the majority of the colonies a basis exists upon which gradual improvements may be made in the direction of assimilating the various registration systems in force to that in operation at home. Of the desirability of such an assimilation there cannot be the slightest doubt; and we would urge upon the colonial authorities to keep this steadily in view. It ought to be not only possible, but perfectly easy to institute a comparison between home and colonial death-rates. To this end the deaths at home and abroad must be recorded on a uniform plan, and the numbers of the population out of which the deaths occur must be distinctly stated. And as registration at home is largely indebted for its proper working to the co-operation (always heartily given) of the medical profession, so it must be abroad; and we commend therefore to our colonial brethren the improvement of their registration systems as a matter in every way worthy their attention.

Correspondence.

"Audi alteram partem."

VACCINATION AND SYPHILIS.

To the Editor of THE LANCET.

SIR,—“Your Reviewer” is a very funny fellow. To parody a description that has become historical, his argument is a jest, his physiology is a jest, everything in his letter is a jest,—but the jokes! Vaccinal syphilis, however, is no matter for jesting; and a joker of jokes without a fragment of a case, is a spectacle analogous to the “good man struggling with adversity.” I take the liberty to blow away the chaff from your reviewer's letter, and to see whether there may, peradventure, be a particle of residual grain.

He describes it as a “Sweeping statement, as yet unconfirmed,” that Hutchinson's disease is of frequent occurrence in families where the evidence of parental syphilis is altogether wanting. I hope the writer knows more about vaccination than he does about “Hutchinson's disease.” So frequent is the latter in cases in which parental syphilis cannot be traced, that many observers have sought a way out of the difficulty by denying, or at least doubting, that the disease is syphilitic at all. To my mind, the evidence on this point is sufficiently strong to justify belief, and to be acted upon in practice. I entertain no doubt that the group of symptoms referred to is a late form of syphilis, frequently due to inheritance. If so, it is certain that the cases in which inheritance cannot be traced are also syphilitic. It is

further certain that they must be examples of syphilis in the same “late form;” and that they cannot be due to direct infection, from primary or secondary disease, through any ordinary channel. Either they are due to inheritance which cannot be traced (and this will account for some cases); or else we must find some other channel through which the virus may be introduced into the system at such a stage of its own development, and in such a way, as to produce not primary or secondary or tertiary syphilis, but the above-mentioned “late form,” or the very series of changes normally due to inheritance. Your reviewer fails to discriminate between “constitutional syphilis” as a whole, and the particular variety in question; and I think it will puzzle him to suggest any channel by which this variety may be accidentally introduced. His suggestion of atavism does not meet the case, because in such instances the inheritance would be easily traceable.

The “parallel between the vaccine virus and the semen” is not mine, but your Reviewer's. I drew no parallel whatever. It has been said, and some people have even believed, that the fluid of the vaccine vesicle was incapable of being contaminated by the state of the system. I argued that the contamination of the semen was a proof of contamination of the body as a whole, and that there was no reason for believing that from this contamination the contents of the vesicle would be exempt.

The fact is, that we cannot at present set any precise limit to the action of the syphilitic virus. That vaccine matter taken from a syphilitic child, and inserted into a healthy one, should produce remote syphilitic changes, would appear to be, *a priori*, not only probable, but inevitable. The difficulty has been that such changes have eluded observation. I contend that they may possibly be discovered in some cases of Hutchinson's disease, and that their dependence upon vaccinal syphilis may have been overlooked, simply because it is only very lately that Hutchinson's disease has been discovered to be syphilitic. Moreover, its most marked sign is not displayed until the appearance of the permanent incisors, and the lapse of time then places impediments in the way of inquiry. Herein lies my reply to Dr. Turner, who left this neighbourhood, I think, in 1865. Mr. Hutchinson's book on inherited syphilis was published in 1863. It is fair to assume that prior to 1863, Dr. Turner would not have recognised the syphilitic character of the inherited disease, and would have called it struma. I very much doubt whether, since 1863, he has ever had leisure or opportunity to investigate, as I have done over and over again, the details of the parental and personal histories of such patients, so as to separate the cases of undoubted inheritance from others. He will pardon me, therefore, for attaching very little value to his great experience, gained, as it mostly was, at a time when he could not have been prepared to understand all that it taught. Moreover, his line of argument seems to me to be essentially unsound. Trousseau gave a *coup de grâce* to the practice of reasoning from “*J'ai vu*,” but those who deny vaccinal syphilis often commit a worse atrocity. They reason from “*Je n'ai jamais vu*!”

In further opposition to Dr. Turner, I may cite the opinion of my partner, Mr. Gregory, for many years certifying surgeon to the factories of the district. He assures me that the Hutchinsonian formation is displayed by many children whose parents show no signs of syphilitic infection.

There is, however, another side to Dr. Turner's experience. He knew the people of his district too well to take lymph from diseased children; and hence, if he saw no vaccinal syphilis, it may have been because his personal knowledge saved him from disseminating it. This is the safeguard which the wisdom of the Legislature has withdrawn. Children are taken to the public vaccinator, who cannot tell whether they are syphilitic or not.

I could say a good deal about the Vaccination Acts, but time and space forbid. I will content myself with correcting a curious error of the press in the last paragraph of your Reviewer's letter. For “public good,” we should obviously read “official convenience.”

I am, Sir,

Your obedient servant,

ROBERT B. CARTER.

[We append the following note from “OUR REVIEWER.”—Ed. LANCET.]

“I am grieved that I should have misapprehended the object of Mr. Carter's first letter. I did not do him the injustice of supposing that he would consider vaccinal syphilis a matter for jesting, but it seemed to me that he might be dealing jestingly with certain loose habits of thought concerning the subject. I willingly defer to Mr. Carter's judgment respecting Hutchinson's disease, but I nevertheless seek for some measure of that frequency with which, as he avers, the malady occurs in families where there

is no evidence of parental syphilis. Until it is known what Mr. Carter actually means by the 'frequent occurrence' of Hutchinson's disease under the circumstances noted, the signification to be attached to his words 'common occurrence' in respect of vaccinal syphilis is not very clear. I used the term 'constitutional syphilis' in reference to cases of quasi-inherited syphilis as more accurately expressing a fact, believing that the context would guard from misapprehension. I do not pretend to set a limit to the action of the syphilitic virus, but I contend that we are not warranted in extending the limit indefinitely or more rapidly than a careful observation teaches. I have no desire to quarrel with Mr. Carter's notion on the probability or inevitability of the vaccine matter from a syphilitic child, when inserted into a healthy one, producing remote syphilitic changes, so long as he admits that at present the discovery of these changes and of their dependency upon vaccinal syphilis is only a possibility. I maintain, however, that until something more definite is shown than the possibility of remote syphilitic changes being brought about by vaccine matter taken from a syphilitic child, Mr. Carter is not justified in stating that syphilitic infection by vaccination is of 'common occurrence.' The question which he has raised is of great interest and moment, but it is to be solved only by observation. It is a question concerning which, to use a happy sentence of Mr. Hutchinson's in one of his papers on inherited syphilis, 'speculations, without positive facts to build on, are worse than useless.' When Mr. Carter has removed the question from the field of speculation to that of ascertained fact, then it will be time enough to ask in what fashion it should influence legislative provisions for vaccination.

"The error of the press in the last paragraph of my letter, to which Mr. Carter should have referred, was in the name of that illustrious poet, patriot, and politician, Mr. Birdofredum Sawin, which was unfortunately maimed by the compositors; and not in the words which he inadvertently instances.

"YOUR REVIEWER."

THE WINDSOR VOLUNTEER REVIEW.

To the Editor of THE LANCET.

SIR,—I am sure the Volunteer medical officers throughout the kingdom must feel indebted to you for the remarks in your journal of last week respecting the mismanagement at Windsor. Here is one little incident that fell under my own observation. Whilst my Corps was preparing for the march-past, the adjutant called my attention to a volunteer lying down in the park at some little distance, whom I found exhausted, and in a fainting condition, caused by the heat and want of nourishment. One of my colleagues joined me, and we administered some restorative; but no water was to be had, nor was there any orderly or ambulance in sight. The man revived a little, but it was not until after a lapse of at least ten minutes that an ambulance arrived and took him away.

I am, Sir, your obedient servant,

June, 1868.

A VOLUNTEER SURGEON.

DUBLIN.

(FROM OUR OWN CORRESPONDENT.)

IN my last letter I discussed one of the grounds upon which it has been advocated that a new charter should be sought for our College of Surgeons, viz., with the view of separating the school from the college, and I stated that one of the arguments urged in support of this separation is the tax that the support of the school is upon the funds of the college. This in round numbers I stated to be something like 300*l.* per annum. Last year the actual sum was 261*l.* 4*s.* 3*d.* But of this the school, as a school, can only be fairly charged with the laboratory expenses, 60*l.*, and the sum paid by the college for advertisements, 31*l.* 10*s.*, in all 91*l.* 10*s.*, inasmuch as the other items were sums that should be expended on the premises for repairs, maintenance, and support, were all the lecture rooms closed, as manifestly the premises, originally constructed at great expense, could not be allowed to run to ruin. I shall now proceed to consider the other reasons upon which the application for a new charter is advocated, and this portion of my letter will possess greater interest for your English readers, especially at the present moment when your elections at the College of Surgeons are

taking place, inasmuch as I shall have to describe the manner in which our elections are conducted, which will afford them an opportunity of contrasting the works of procedure at their own and our college. The other important arguments are that in the preparation of a new charter provisions could be introduced for allowing country Fellows to vote by proxy, and for permitting our Examiners to be elected from amongst the Professors and Lecturers in the several schools, gentlemen who, as matters now stand, are not permitted to undertake these duties. To enable you to understand these points, I must inform you that the entire government of the College is entrusted to the President, Vice-President, and Council (19 in number), who are only elected for one year, and who go out of office on the first Monday in June, but who are all eligible for re-election, except in the case of President and Vice-President; these officers, so far as I can remember, never having been re-elected to fill the same posts, the rule being that the Vice-President is, through courtesy, elected without opposition President, and in the vast majority of instances the outgoing President is brought back upon the Council; the contest being for the office of Vice-President, and for that of Councillor. Now, on the first Monday in June every office is virtually vacant; it is competent for the College, should it so seem fit to them, to elect twenty-one Fellows who never had a seat at the Council, one as President, one as Vice-President, and the other nineteen as Councillors, and so start with new blood indeed. 'Tis true that they never have seen fit to adopt so sweeping a change, but they have the power in their own hands to do so, and at some of the elections held a few years ago, the College made some radical changes in the Council, for reasons unnecessary to allude to here, more particularly—since then however, unless in the case of a vacancy by death or resignation, the outgoing Council has been almost invariably re-elected, the President being brought back to the vacancy caused by the promotion of some member of council to the post of Vice-President, the pretty generally expressed opinion of the Fellows of the College being that it is advisable that a gentleman should serve some time on the Council, so as to learn the business of the College and thereby qualify him for discharging the duties of Vice-President and of President. By charter a Member of Council is prohibited from filling the office of examiner in the College. There are three distinct Courts of Examiners, one for the Preliminary Examination, composed of three Examiners, who need not necessarily be Fellows of the College, and which court in fact is at present composed of one of the Fellows of Trinity College, Dublin; an LL.D. of the same University, neither of whom are medical men, and of an ex-scholar of the same University who is a Fellow of the College of Surgeons. Next we have the Midwifery Court, also three in number, and then we have the Court of Examiners for the Licence, or the Letters Testimonial as it is called, and for the Fellowship of the College (seven in number) all of whom must be Fellows of the College, but by Charter are prohibited from being either Lecturers in Schools, or Councillors of the College, so to accept a seat on the Council necessarily debars one from the more lucrative post of Examiner. The election of all the Examiners is vested in the Council, takes place on the first Tuesday in May, and is conducted in the following manner:—seven members of Council are selected by ballot, and to them is entrusted the duty of determining who shall be the Examiners for the ensuing twelve months; great care is taken in making the selection, and, as a general rule, the Examiners appointed have given cause for unmixed satisfaction to the College for both ability and impartiality. Some years ago, an opinion was expressed by the Council that it would be desirable that the Examiners should not hold office longer than for five years, but in practice this is not attended to, many of the present Examiners having exceeded this period by many years. From this brief statement you will perceive that the government of the College virtually is vested in the Fellows, as they have annually the power of electing the Council, which Council subsequently elects the Examiners, Professors, &c., and have for three years of office entire control of the affairs of the College, subject, however, to the risk of not being re-elected should they run counter to any expressed wish of the assembled Fellows at the annual meeting held, to receive the report of the Council for their year of office, which meeting is held a fortnight before the annual meeting for election in June, or at any extraordinary meeting of the Fellows, for which the charter has made ample provision by making it incumbent on the President to convene a general meeting of the Fellows at large, on the written requisition to that effect, presented to him and signed by any twelve Fellows of the College. On more than one occasion the Fellows have availed themselves of this privilege, and it is not very long ago since they showed their power by compelling the Council to adopt measures to which the majority of them were